NCCP 00415 · VERSION 5

Your treatment, explained

NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma. · NCCP 00415 v5 · Document revised 6 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma

Reference calendar for daily medicine and reviews

Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending

ACT ON SYMPTOMS, NOT THE CALENDAR

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

Your treatment and urgent-contact card is supplied separately and in the wallet-card section of the complete handbook.

Sources: nccp

Reference contents

YOUR COMPLETE REFERENCE

Read what you need, when you need it.

Start with the overview, explore the cycle, or go straight to a question. Urgent advice is always available.

How each drug works
The overall treatment plan
Your 28-day timeline
Baseline and cycle-specific checks
Whole treatment, scans and checks
Tablets, visits and practical planning
Understanding your diagnosis and treatment goal
Your medicines and the overall plan
Taking treatment and preparing for visits
Tests: what is required and what each result means
What to expect between visits
Skin checks, sun protection and optional photographs
Pregnancy, fertility and protecting others
Questions, decisions and practical support
Follow-up and your continuing record
Supportive medicines
Understanding the evidence
Questions to ask
Sources

UNDERSTAND YOUR TREATMENT

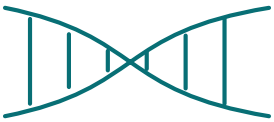
Medicines in this regimen

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

ORAL BRAF INHIBITOR

Dabrafenib

Tafinlar



Blocks altered BRAF signalling

A BRAF V600 change can keep the cell-growth pathway switched on. Dabrafenib blocks the altered BRAF protein. It is paired with a MEK inhibitor to act on the same pathway at another point. The BRAF result comes from tumour testing; it does not automatically mean a hereditary condition.

BRAF V600 result → BRAF blocked → Growth signal reduced

What to watch for

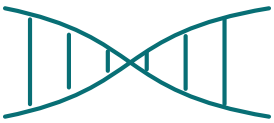
Fever needs the written hold/contact plan. Check other medicines and contraception because interactions can matter.

Sources: nccp · nccn-patient

ORAL MEK INHIBITOR

Trametinib

Mekinist



Blocks downstream MEK signalling

MEK carries growth signals further along the pathway below BRAF. Trametinib blocks MEK alongside dabrafenib. Normal heart, eye, lung and other tissues can also be affected, which is why scheduled tests and prompt symptom reporting matter.

MEK signal blocked → Works with dabrafenib → Heart / eye monitoring

What to watch for

Attend cardiac tests. New eye symptoms, cough, breathlessness, swelling or severe diarrhoea need prompt review.

Sources: nccp · nccn-patient

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

The anatomical side-effect map follows.

REGIMEN-SPECIFIC SIDE EFFECTS

Your anatomical side-effect map

A picture of side effects

Match each symptom box to its numbered body location. Tell your team about new symptoms.

1 Fever / chills

Both medicines

At 38°C or above, hold both medicines and contact the team under your fever plan. Infection and dehydration must also be considered.

3 Heart / circulation

Trametinib / combination

Keep ECG and heart ultrasound checks. New breathlessness, swollen ankles, palpitations or chest pain needs prompt assessment.

5 Liver

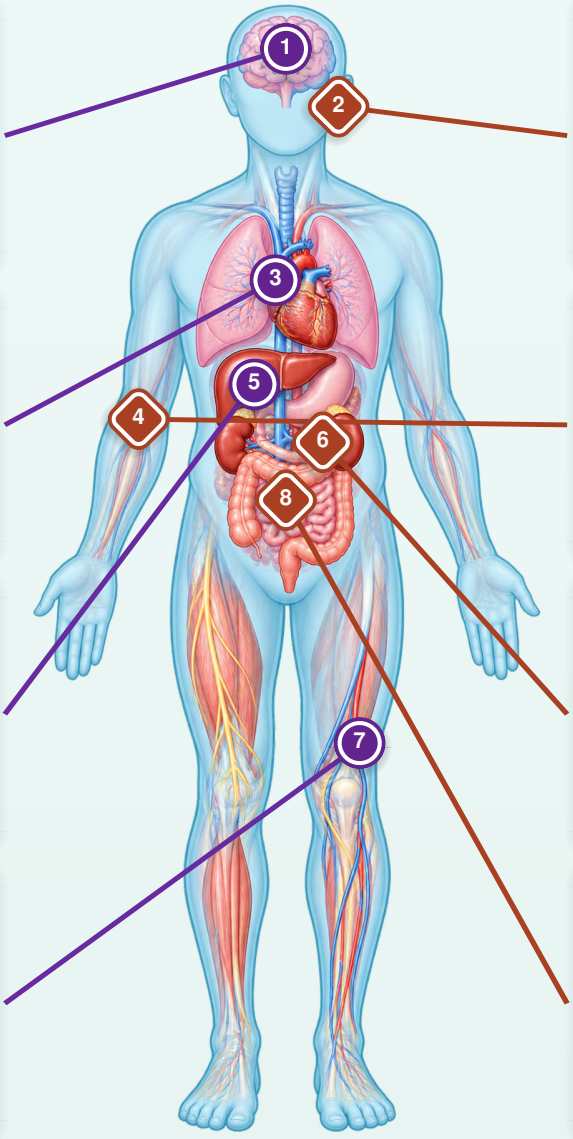
Both medicines

Liver changes may first appear in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

7 Muscles / joints

Both medicines

Aches can occur. Severe muscle pain or weakness, especially with dark urine, needs urgent advice and possible blood tests.



2 Eyes / vision

Dabrafenib / trametinib

Report blurred vision, eye pain, redness, light sensitivity or a change in sight promptly; some eye problems need urgent specialist treatment.

4 Skin / new lesions

Both medicines

Rash and skin sensitivity may occur; new changing spots need review. Blistering, peeling or mouth sores with a rash needs urgent help.

6 Kidneys / fluids

Both medicines / fever

Fever, vomiting or diarrhoea can cause dehydration and kidney problems. Call for reduced urine, dizziness or inability to keep fluids down.

8 Stomach / bowels

Both medicines

Tell the team about persistent diarrhoea, vomiting or pain. Severe abdominal pain, blood or black stools needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

THE BIGGER TREATMENT PLAN

The overall treatment plan

12-MONTH PLANNED COURSE

Adjuvant oral treatment

Planned 12 months; regular reviews

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

The treatment setting determines duration; a reference calendar is not a prescription or a reason to restart a held medicine.

Sources: nccp

BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00415 v5: current regimen source · NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Adjuvant dabrafenib and trametinib after removal of BRAF-mutated stage III melanoma: treatment and checks

NCCP 00415b: adjuvant treatment after complete resection of stage III melanoma with a BRAF V600 mutation. This route is distinct from treatment of unresectable or metastatic melanoma.

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00415	Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/ liver tests, ECG, echocardiogram, skin examination and eye assessment when indicated.	Blood count, kidney/ liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.
Cancer, skin and daily function NCCP 00415	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up depends on stage, treatment and symptoms. Your team records the skin/ lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.
Medicines and continuing precautions NCCP 00415	Review all medicines and the pregnancy/ fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

What each check is for

1

Adjuvant oral treatment

12-month planned course

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00415

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40

Day 1 → Day 28: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Treatment timing: displayed days 1–28

Treatment or check	Days in this window	What happens
Daily oral treatment	Days 1–28	Both medicines daily; adjuvant course planned for 12 months with regular safety reviews.

Treatment or check	Days in this window	What happens
Safety reviews as scheduled	Days 1–28	Keep the specific blood, organ and skin-review appointments; they do not all follow the same interval.
Symptoms and support	Days 1–28	Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

Timing note 1: Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Timing note 2: The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

Timing note 3: Advanced and adjuvant treatment are distinct; do not apply a 12-month endpoint to all patients.

Timing note 4: Fever holds both drugs; other toxicities may have component-specific rules. The treating team decides restart and dose changes.

Timing note 5: The historical PDF contains adjuvant00415b while the reviewed catalogue excerpt lists00415a; current source reconciliation must resolve/record this difference.

Timing note 6: Regular skin reviews are required; ask the team to confirm the interval and keep the recorded appointments.

> **Why these bands stay broad:** This calendar shows the pattern. Your team supplies actual dates, prescribed doses and any hold or restart instructions. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00415 v5: current regimen source ↗

> **There are no guaranteed "safe days".** The prescribed dose, treatment decision and local urgent-contact plan take priority. Call 112 or 999 for severe breathing difficulty, severe chest pain, collapse, a new seizure, sudden weakness or speech difficulty, or uncontrolled bleeding. Follow your oncology unit's written urgent-contact plan for other new or worsening symptoms; do not wait for the next appointment.

Complete timing and symptom notes

Day 1–28: Every day in the reference calendar

Both medicines daily; adjuvant course planned for 12 months with regular safety reviews. Use the written medicine plan; report new symptoms and keep tests even during a treatment hold.

PRACTICAL TREATMENT GUIDE

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Taking or receiving treatment

Swallow the prescribed capsules/tablet whole with water. Take each dose at least one hour before food or at least two hours after food. Take the once-daily trametinib dose at the same time each day, alongside either the morning or evening dabrafenib dose according to your written plan. Keep the prescribed storage instructions.

STEP 2

Missed, vomited or held treatment

The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

STEP 3

Appointments and supply

Arrange dates, medicine supply, transport and any needed interpreter or carer support. Ask whether bloods are taken in the oncology unit or elsewhere and how the result reaches the prescriber.

STEP 4

Paper copies and contact information

Keep this complete handbook, the three-page summary and the wallet card together. The QR code is optional. Ask the team to add your actual contacts and print a revised plan when treatment changes.

Sources: NCCP 00415 v5: current regimen source. Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending.

MAKE THE PLAN PRACTICAL

Support around the medicines

1 Fever action plan

If temperature reaches 38°C or higher, withhold both dabrafenib and trametinib and contact your oncology team promptly for assessment. For recurrent episodes, the team may advise withholding at the first symptom. Restarting needs the team's plan after at least 24 hours without symptoms; do not use this as an automatic self-restart rule.

2 An individual symptom plan

Ask which anti-sickness or other symptom medicines are prescribed, when to take them and when to call if they do not help. Do not start a borrowed medicine or alter treatment to manage a side effect yourself.

3 Your urgent contacts

Keep your actual oncology, out-of-hours and pharmacy contacts on the wallet card. Example numbers on a template must be replaced before you use it.

4 Pregnancy and fertility precautions

Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

5 Eating, activity and practical help

Report falling intake, weight loss, pain or fatigue early. The team can tailor nutrition, activity, wound, psychological and practical support to your needs.

Sources: nccp · nccn-patient

TREATMENT REFERENCE · 01

Understanding your diagnosis and treatment goal

Use these questions with your own pathology report and agreed treatment plan.

What the diagnosis says

Keep the pathology summary, original skin site, stage and details of any lymph-node involvement together. Breslow thickness and ulceration describe features of the original melanoma; they are different from the stage. If the diagnosis arose in the eye or a mucosal surface, ask whether this cutaneous-melanoma information applies.

Sources: nccn-patient

BRAF and other tumour tests

A BRAF V600 result helps decide whether the BRAF/MEK targeted medicines in this guide are suitable. This is usually a test of the tumour, not proof of an inherited family condition. Keep the actual result and test date; ask about genetic counselling only when your personal or family history makes that relevant.

Sources: nccn-patient

Surgery, radiotherapy and medicine have different roles

Record what surgery removed and whether the present medicine is postoperative treatment to lower recurrence risk or treatment of cancer that remains or has spread. Radiotherapy, stereotactic treatment or another operation may have a separate role; their timing and side effects need their own plan.

Sources: nccn-patient

Stage and response are different

Stage describes the extent of melanoma at diagnosis or a later reassessment. A response assessment asks how known cancer has changed with treatment. In the adjuvant setting there may be no visible tumour to measure: surveillance looks for recurrence while safety checks look for medicine effects.

Sources: nccn-patient

A new skin change still needs review

Continue to report a new or changing mole, a non-healing lesion, a new lump or an unexplained persistent symptom. Do not assume every new lesion is melanoma, and do not use a photograph or a good response at one site to rule out a problem elsewhere.

Sources: nccn-patient

TREATMENT REFERENCE · 02

Your medicines and the overall plan

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

The schedule in this setting

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

Sources: nccp

How the treatment works

A BRAF V600 change can keep the cell-growth pathway switched on. Dabrafenib blocks the altered BRAF protein. It is paired with a MEK inhibitor to act on the same pathway at another point. The BRAF result comes from tumour testing; it does not automatically mean a hereditary condition. MEK carries growth signals further along the pathway below BRAF. Trametinib blocks MEK alongside dabrafenib. Normal heart, eye, lung and other tissues can also be affected, which is why scheduled tests and prompt symptom reporting matter.

Sources: nccp

The role of earlier or later treatment

Record surgery, radiotherapy and other anticancer medicines separately, including dates and the specialist responsible. This schedule does not automatically include or authorise another treatment. Ask whether any overlap changes tests, symptoms to report or the timing of appointments.

Sources: nccn-patient

Pauses and stopping

A safety pause is an active treatment decision. Record which medicine is paused, the reason, the tests or recovery needed and who may authorise restarting. Do not restart from an old prescription or reset the history when a new phase begins.

Sources: nccp

TREATMENT REFERENCE · 03

Taking treatment and preparing for visits

Keep the written medicine plan beside this guide; it records your own dose and dates.

Taking or receiving the medicine

Swallow the prescribed capsules/tablet whole with water. Take each dose at least one hour before food or at least two hours after food. Take the once-daily trametinib dose at the same time each day, alongside either the morning or evening dabrafenib dose according to your written plan. Keep the prescribed storage instructions.

Sources: nccp

Missed, vomited or held treatment

The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

Sources: nccp

Other medicines and supplements

Show the pharmacist a current list of prescriptions, over-the-counter medicines, vitamins, herbal products and supplements. Include anticoagulants, heart medicines, steroids and recent antibiotics. Check before adding, stopping or changing anything; an apparently harmless product can alter treatment or confuse a symptom assessment.

Sources: nccp

Practical preparation

Arrange medicine supply, transport, interpreter or carer support before it is needed. Bring the alert card, symptom record and questions. Ask which appointments require someone to accompany you and how to obtain urgent advice outside normal hours.

Sources: nccn-patient

TREATMENT REFERENCE · 04

Tests: what is required and what each result means

Safety tests, tumour tests and response scans answer different questions.

Before treatment

Confirm the BRAF V600 result, complete resection of stage III melanoma and the postoperative treatment goal. Full blood count, kidney/liver tests, ECG, echocardiogram, skin examination and eye assessment when indicated.

Sources: nccp

During treatment

Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required; your oncology team will confirm and record the interval. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan.

Sources: nccp

Scans and tumour markers

Ask the team to write the next assessment and who will arrange it. Bloods used to monitor kidneys, liver, hormones or muscle are not automatically measures of cancer response. There is no universal tumour-marker blood schedule for these skin-cancer regimens; a scan or extra blood test is requested for a specific clinical reason.

Sources: nccp

After a pause or finish

Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.

Sources: nccp

Getting the result

For each test, record its purpose, due date, result and action. Agree how results reach you and who checks a result from another hospital. If an appointment or result has not arrived, contact the named service rather than assuming no news means no action is needed.

Sources: nccn-patient

TREATMENT REFERENCE · 05

What to expect between visits

You can feel well and still need the planned checks. New symptoms deserve attention even on a day without an appointment.

Your usual pattern

Record the start and change in symptoms, temperature if advised, bowel pattern, drinking, urine and the effect on normal tasks. A simple few-line record is enough. Use it to describe a change rather than trying to grade or diagnose a toxicity yourself.

Sources: nccp

When to call

Fever is a particular concern. At 38°C or above, withhold both medicines and contact the oncology team promptly under the written fever plan; recurrent episodes may require action at the first symptoms. Infection and dehydration also need assessment. Restart only when the team tells you. Urgent help is also needed for vision change, severe rash, chest pain, breathlessness, bleeding or severe muscle symptoms.

Sources: nccp

Eating, drinking and activity

Tell the team early about nausea, taste change, a sore mouth, diarrhoea or falling weight. Ask for an individual fluid and food plan if heart, kidney or other conditions limit intake. Gentle activity and planned rest can help maintain function; adjust to symptoms and ask for support when fatigue limits daily life.

Sources: nccn-patient

A held dose and a new phase

The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

Sources: nccp

TREATMENT REFERENCE · 06

Skin checks, sun protection and optional photographs

This complements clinical assessment and does not replace it.

Protect your skin

Use shade, protective clothing, a hat and broad-spectrum sunscreen of at least SPF 30, and avoid tanning beds. Reapply sunscreen according to its instructions. Protection matters even when it is cloudy; choose a practical approach that lets you remain active safely.

Sources: nccn-patient

Know the pattern of your skin

Ask how to examine your skin, scars and relevant lymph-node areas and how often your own team recommends doing so. A mirror or a trusted helper can make hard-to-see areas easier to check. Report a new or changing lesion, a non-healing sore or a new lump rather than waiting for a routine visit.

Sources: nccn-patient

Photographs are optional

If useful, keep a dated photograph with consistent lighting and a size reference, following your team's advice. Photographs are optional and do not diagnose a cancer or replace examination. Keep them privately; this printable passport does not require you to upload images or other personal health information.

Sources: nccn-patient

Wounds, nails and sensitive skin

Use the dressings and skin-care plan recommended for your lesion and treatment. Ask before applying new creams or remedies to an open or treated area. Worsening pain, discharge, redness, bleeding or loss of function needs advice; routine skin-care instructions are not a substitute for assessment.

Sources: nccn-patient

TREATMENT REFERENCE · 07

Pregnancy, fertility and protecting others

Discuss these precautions before treatment even if pregnancy is not currently planned.

Medicine-specific precautions

Discuss fertility and pregnancy precautions before starting. Use an effective alternative to hormonal contraception during treatment and for two weeks after dabrafenib and 16 weeks after trametinib; dabrafenib can make hormonal contraception less effective. The team must confirm the method and the later applicable end date for the combination. Do not breastfeed during treatment; ask when it is safe to resume. Report a possible pregnancy promptly.

Sources: nccp

Planning ahead

Ask early about fertility preservation, sexual health and symptoms affecting intimacy. Tell the team about any chance of pregnancy, breastfeeding or a partner's pregnancy so that the correct medicine-specific advice can be given. Keep the precautions and their end dates in the written plan.

Sources: nccp

Safe handling and supply

Do not share anticancer medicines. Keep oral medicines in their original labelled packaging, away from children and anyone who could take them accidentally. Return unused medicines to the supplying pharmacy and follow its handling advice. A treatment hold does not remove pregnancy precautions.

Sources: nccp

TREATMENT REFERENCE · 08

Questions, decisions and practical support

Your priorities and the burden of treatment belong in the discussion.

Benefits and uncertainties

Ask what evidence applies to your diagnosis, stage, biomarker and treatment setting, what outcome the team hopes to improve, and what may happen without this treatment. A trial average cannot predict an individual result; a benefit measured in one setting should not be transferred to another.

Sources: nccn-patient

Alternatives and review points

Discuss other suitable treatments, clinical trials if relevant, local treatment options and symptom-focused care. Ask when the decision will be reviewed and what would prompt a change. Supportive and palliative care can help with symptoms and quality of life alongside anticancer treatment.

Sources: nccn-patient

Help with daily life

Tell the team about work, transport, caring responsibilities, cost concerns, body image, anxiety or sleep. Ask which nurse, social worker, dietitian, rehabilitation professional or support organisation can help. Use resources relevant to your country and service; a US guide may describe different access or funding.

Sources: nccn-patient

Shared understanding

Ask someone on the team to check that you can explain the medicine schedule, next tests and urgent-contact route in your own words. An interpreter, large-print copy or a trusted supporter can help; none requires you to use a smartphone.

Sources: nccn-patient

TREATMENT REFERENCE · 09

Follow-up and your continuing record

Keep this record when treatment is paused, changed or completed.

Your follow-up plan

Follow-up depends on stage, treatment and symptoms. Your team records the skin/lymph-node examination plan, whether body or brain imaging is needed, and which service owns each appointment. A general patient booklet does not set a universal scan interval for every patient.

Sources: nccn-patient

Continuing medicine precautions

Continue skin surveillance for six months after stopping as specified by the regimen, and arrange the ongoing melanoma follow-up plan. Review symptoms, heart/eye concerns and contraception instructions after treatment.

Sources: nccp

Treatment summary for other clinicians

Record the medicines and last dates, important reactions, any long-term hormone replacement or steroid taper, and the responsible oncology contact. Show this information in urgent care and to other specialists. A different treatment phase does not erase earlier toxicity.

Sources: nccp

The complete paper version

This handbook can be used without a phone: keep the schedule, symptom advice, medicine list, tests and contacts together. The QR code is an optional route to the matching regimen information. Ask the team to print updates when your plan changes and keep the latest dated copy.

Sources: nccn-patient

What the evidence means for your treatment

The evidence discussion must match the diagnosis, biomarker and treatment setting shown in this guide.

Ask which outcomes matter in your setting and what benefit is realistic for you. No survival percentage from another regimen, dose combination or disease setting is assigned here.

Sources: nccp · nccn-patient

PREPARE FOR THE CONVERSATION

Questions worth asking

What is the aim of my treatment?

After eligible stage III melanoma has been completely removed, dabrafenib and trametinib are offered to reduce the chance of recurrence. The expected benefit, alternatives and treatment burden need an individual discussion; treatment cannot guarantee that melanoma will not return.

Is there a fixed number of cycles?

Take dabrafenib twice daily, around 12 hours apart, and trametinib once daily, every day unless the team instructs a hold. The adjuvant course is planned for 12 months, unless stopped earlier for recurrence, side effects or another clinical reason. A four-week review is not a drug holiday or one of 12 chemotherapy cycles.

What happens if treatment is held?

The missed-dose windows differ: take missed dabrafenib only when more than six hours remain before the next dose; take missed trametinib only when more than 12 hours remain. Otherwise skip it. Do not double up. Do not take an extra trametinib dose after vomiting; contact the team or pharmacist about any vomited or uncertain dose.

When are my next tests?

Blood count, kidney/liver tests monthly. ECG and echocardiogram at four weeks, then every 12 weeks and after dose modification. Regular skin reviews are required: ask the team to document the interval because the source gives monthly and 60-day instructions in different sections. Head/neck examination every three months and chest CT every six months are specified surveillance checks; disease-response imaging follows the individual plan. Your team adds the actual dates.

Can I use this without a smartphone?

Yes. The full handbook, three-page summary and wallet card are printable. The QR code is optional; keep actual contacts and written appointments on the paper copy.

> **Try explaining the plan back:** Can you explain your treatment setting, medicine schedule, next tests, the symptoms that need urgent advice and the number you would call?

SOURCE TRANSPARENCY

Where this information comes from

Regimen schedule: NCCP 00415 v5. Medicine explanations and safety information: sources below. The timeline is an educational illustration, not a measured risk curve or an individual prediction.

- NCCP 00415 v5: current regimen source
- NCCN Guidelines for Patients: Melanoma, 2026 - supplied education reference

SACTCheck · Developed by Dr Paul O'Brien · Sources checked 2026-11-06.
This information supports your treatment discussion. Your oncology team confirms your personal treatment and urgent-care plan.

Written source addresses

https://healthservice.hse.ie/documents/6732/415_Trametinib_and_DabrafenibTherapy.pdf

https://www.nccn.org/patients/guidelines/content/PDF/melanoma-patient.pdf